



ESSAY

Threats to Contraception

Deborah Tuerkheimer*

Access to contraception has become newly critical since the Supreme Court decided *Dobbs v. Jackson Women's Health Organization*.¹ By eliminating a federal constitutional right to abortion, the Court raised the already urgent stakes for those seeking to control their reproductive lives, and for sexually active women in particular.² At the same time, *Dobbs* destabilized the contraceptive right first recognized more than fifty years earlier in *Griswold v. Connecticut*.³ While the *Dobbs* majority purported to embrace an abortion exceptionalism that would cabin its holding to pregnancy termination,⁴ the Court's selected "history and tradition" test could not but implicate other entrenched constitutional rights—including contraception.⁵ In a separate

* Class of 1967 James B. Haddad Professor of Law, Northwestern University Pritzker School of Law. I am grateful to Sarah Lawsky and Janice Nadler for insightful comments on an earlier draft, to Jessica Valenti for her crucial newsletter, *Abortion, Every Day*, and to editors at the *Stanford Law Review Online*, and to Hannah Repke and Sarah Ryan in particular, for outstanding suggestions that meaningfully improved the Essay. Tom Gaylord, Associate Law Librarian for Scholarly Communications, and Areeb Asif, J.D. 2025, provided excellent research assistance. Generous financial support was enabled by the Northwestern Pritzker School of Law Faculty Research Program.

1. 142 S. Ct. 2228, 2284-85 (2022).

2. See *Nomination of Ruth Bader Ginsburg, To Be Associate Justice of the Supreme Court of the United States: Hearing Before the S. Comm. on the Judiciary*, 103d Cong. 207 (1993) (statement of Ruth Bader Ginsburg) ("The decision whether or not to bear a child is central to a woman's life, to her well-being and dignity. . . . When Government controls that decision for her, she is being treated as less than a fully adult human responsible for her own choices.").

3. 381 U.S. 479, 485-86 (1965) (establishing the constitutional right of married couples to use contraception); see also *Eisenstadt v. Baird*, 405 U.S. 438, 454-55 (1972) (extending the contraceptive right beyond the confines of marriage).

4. *Dobbs*, 142 S. Ct. at 2227-28 ("Nothing in this opinion should be understood to cast doubt on precedents that do not concern abortion.").

5. For example, the Comstock Act, enacted in 1873, criminalized the use of the postal service to send any "article or thing designed or intended for the prevention of conception or procuring of abortion." Comstock Act, ch. 258, § 2, 17 Stat. 598, 599 (1873) (emphasis added) (codified as amended at 18 U.S.C. §§ 1461-1462 (2024)); see Reva B. Siegel & Mary

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opinion emphasizing his disagreement with the very notion of substantive due process, Justice Thomas was forthright in his call for reconsideration of *Griswold*.⁶ Situated alongside the majority's logic, Justice Thomas's concurrence casts grave doubt on the future of a constitutional right to contraception.

But the Supreme Court is not the most immediate threat to birth control.⁷ This Essay argues that attacks on contraception do not depend on a direct judicial repudiation of *Griswold*. Rather, an era of contraceptive siege is already upon us. Access is diminishing through three incremental and mutually reinforcing dynamics: the spillover effects of *Dobbs* that shrink service capacity and enable the conflation of abortion and birth control; the ascendancy of parental- and conscience-based claims that authorize new gatekeeping; and a misinformation-driven cultural turn against hormonal methods that lays the groundwork for restrictive regulation in the guise of protection. Taken together, these developments jeopardize the availability of contraception even as constitutional doctrine remains, at least for the moment, formally intact.

The subtlety of the ongoing anti-contraception offensive is by design, as contraception is extremely popular with the American public.⁸ Since a frontal assault on access is not politically viable, conservative lawmakers and policymakers must hedge,⁹ sow confusion,¹⁰ or appeal to an altogether distinct

Ziegler, *Comstockery: How Government Censorship Gave Birth to the Law of Sexual and Reproductive Freedom, and May Again Threaten It*, 134 YALE L.J. 1068, 1100-13 (2025) (chronicling the history of Comstock Act enforcement against abortion and contraception). Beginning in the late nineteenth century, nearly half the states enacted so-called mini-Comstock Acts. Siegel & Ziegler, *supra*, at 1081 n.48 (noting that “[m]any such laws went further than the federal statute: twelve made illegal speech about abortion or contraception, for example, while eleven criminalized the possession of information about contraception,” and one—Connecticut—criminalized contraceptive use (emphasis omitted) (citation omitted)).

6. *Dobbs*, 142 S. Ct. at 2301 (Thomas, J., concurring) (“[I]n future cases, we should reconsider all of this Court’s substantive due process precedents, including *Griswold* . . .”).
7. To be clear, *Griswold* may well go the way of *Roe*. See *supra* notes 3-6 and accompanying text. Regardless, the threats I describe here are far more imminent.
8. See Geoffrey Skelley & Holly Fuong, *How Americans Feel About Abortion and Contraception*, FIFTYEIGHT (July 12, 2022, 6:00 AM), <https://perma.cc/9PWS-576S> (detailing the “overwhelming support for legal access to common forms of contraception across party lines”).
9. This tactic is evident in the failure of recent efforts to shore up a contraceptive right at the federal level and in some states. See, e.g., Sahil Kapur, Julie Tsirkin, Frank Thorp V & Kate Santaliz, *Senate Republicans Block Bill to Protect Access to Contraception*, NBC NEWS (June 5, 2024, 1:45 PM PDT), <https://perma.cc/5J4S-E8PJ> (reporting that a vote on the Right to Contraception Act failed largely along party lines, and that “Republicans said it was unnecessary because the use of birth control is already protected under Supreme Court precedent”); Shannon Heckt, *Bill to Protect Right to Use Contraception Stalls in Legislature*, KLFY (Mar. 27, 2024, 3:47 PM CDT), <https://perma.cc/BJ6B-HXZA> (explaining that “[d]espite an amendment being added [to the Bill] making it so that any provider could opt out of prescribing contraceptives for any reason, lawmakers

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cluster of legal norms.¹¹ In the coming months and years, as contraception becomes gradually more imperiled, ensuring access will require sustained challenges to stealthy legal and cultural interventions that, while working in unison, might appear to be unrelated.

In this Essay, I identify the underpinnings of an emerging movement to restrict contraception. Part I describes the spillover effects of *Dobbs*: clinic closures, funding restrictions, and definitional confusion around various forms of birth control. Part II details direct strikes on contraception that capitalize on greater legal receptivity toward, first, parental rights and, second, religious objection. Part III anticipates new regulation under the guise of safety—ground now being seeded by the spread of misinformation around hormonal birth control. Part IV maps the vital connections between these seemingly disparate contraceptive attacks. I conclude with brief thoughts on combating these attacks.

I. *Dobbs* Defeats Access

On its face, *Dobbs* is a decision about abortion. But the holding also has the practical effect of shrinking access to contraception.¹²

One reason is the growing nationwide threat to family planning clinics, which offer a range of reproductive health services, including contraceptive care.¹³ Since *Dobbs*, many clinics have closed, resulting in the decreased availability of birth control.¹⁴ Post-*Dobbs* clinic closures are hardly confined to

wanted more time to think about their votes and to speak to Right to Life about their position”).

10. See *infra* notes 27-39 and accompanying text.

11. See *infra* notes 42-65 and accompanying text.

12. Even before *Dobbs*, low-income women confronted severe and often insurmountable constraints on their ability to access contraception, especially in its most effective forms. For a discussion of these constraints and their particular impact on women of color, see Deborah Tuerkheimer, *Unwanted Pregnancy: Sex, Contraception, and the Limits of Consents*, 110 MINN. L. REV. (forthcoming 2025) (manuscript at 36-41) (on file with author).

13. According to the most recent Planned Parenthood Annual Report, contraceptive care constitutes nearly a quarter of the medical services provided by its clinics, while STI testing and treatment comprise over half the total; abortion services constitute just four percent of the services provided. PLANNED PARENTHOOD, ANNUAL REPORT 2023-2024, at 22 (2024), <https://perma.cc/M32H-7FGB>.

14. See Rachel K. Jones, Candace Gibson & Jesse Philbin, *The Number of Brick-and-Mortar Abortion Clinics Drops, as U.S. Abortion Rate Rises: New Data Underscore the Need for Policies that Support Providers*, GUTTMACHER (June 2024), <https://perma.cc/YVL9-XMGP>; see also *infra* notes 17-20 and accompanying text.

abortion-restrictive states. In less than two years alone, more than thirty clinics have ceased operations in states that allow abortion.¹⁵

These closures reflect a set of mounting challenges to clinics, even in states where the right to abortion is protected. Most recently, the Trump administration has frozen a significant portion of funding under Title X, the federal statute establishing the Family Planning Program (Title X) that provides low-income patients with reproductive health care—including contraception.¹⁶ If the current withholding of federal funds becomes permanent, it would result in more than 834,000 people—30% of patients served annually—losing access to Title X-funded care in a year.¹⁷ In seven states, Title X care would be fully eliminated.¹⁸ On top of federal funding restrictions,¹⁹ clinic operators must contend with states placing limits on contraceptive coverage²⁰ and even eliminating Medicaid reimbursements to clinics entirely.²¹

15. Kate Wells, *Why So Many Clinics That Provide Abortion Are Closing, Even Where It's Still Legal*, NPR (May 16, 2025), <https://perma.cc/JE46-GAUN>.

16. Megan L. Kavanaugh, Nakeisha Blades, Amy Friedrich-Karnik & Jennifer J. Frost, *Trump Administration's Withholding of Funds Could Impact 30% of Title X Patients*, GUTTMACHER (Apr. 8, 2025), <https://perma.cc/EH74-ZMK3>. With respect to the allocation of remaining Title X funds, the Trump administration has recently signaled an interest in promoting pregnancy and “holistic” approaches to fertility. Caroline Kitchener & Sheryl Gay Stolberg, *Under Trump, a New Focus for a Birth Control Program: Helping Women Get Pregnant*, N.Y. TIMES (July 18, 2025), <https://perma.cc/FL7N-WPUB> (“The Trump administration intends to use funds from a decades-old federal program that provides birth control to low-income women to ramp up efforts to help aspiring mothers get pregnant, signaling a shift in policy that will appease both religious conservatives and adherents of its Make America Healthy Again agenda.”); see also Caroline Kitchener, *Trump Administration Decimates Birth Control Office in Layoffs*, N.Y. TIMES (Oct. 15, 2025), <https://perma.cc/X8BY-DJS3> (“The Trump administration has targeted a federal office that oversees a \$300 million family planning program for layoffs, raising fears that it is effectively ending an initiative that provides contraception for millions of low-income women . . .”).

17. Kavanaugh et al., *supra* note 16.

18. *Id.*

19. Brittni Frederiksen, Ivette Gomez & Alina Salganicoff, *Title X Grantees and Clinics Affected by the Trump Administration's Funding Freeze*, KFF (Apr. 15, 2025), <https://perma.cc/EP2S-U3RX>. For other efforts to defund Planned Parenthood, see Katia Riddle, *The GOP Budget Bill Threatens to Defund Planned Parenthood*, NPR (July 4, 2025, 4:16 PM ET), <https://perma.cc/6JEW-AQYP>. See also Planned Parenthood Fed'n of Am., Inc. v. Kennedy, No. 25-cv-11913, 2025 WL 2101940, at *28-29 (D. Mass. July 28, 2025) (blocking the provision of the One Big Beautiful Bill Act that prohibited Planned Parenthood from receiving federal Medicaid reimbursements).

20. See *Insurance Coverage of Contraceptives*, GUTTMACHER (Jan. 2, 2025), <https://perma.cc/9BUE-FJUC> (listing states that exclude coverage for prescription contraception methods and those that allow cost sharing).

21. See *Medina v. Planned Parenthood of S. Atlantic*, 145 S. Ct. 2219, 2234-39 (2025) (holding there is no private right of action to prevent a state from effectively blocking Medicaid
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In the wake of *Dobbs*, clinic operators face daily fallout from the Court's decision to leave the question of abortion to the states. As co-director of the Abortion Care Network explained,

It's gotten more expensive to provide care, it's gotten more dangerous to provide care, and it's just gotten, frankly, harder to provide care, when you're expected to be in the clinic and then on the statehouse steps, and also speaking to your representatives and trying to find somebody who will fix your roof or paint your walls who's not going to insert their opinion about health care rights.²²

Before *Dobbs*, publicly funded family planning clinics provided contraceptive services to one quarter of all U.S. women receiving contraceptive care.²³ As more clinics are forced to shutter their doors²⁴ and those that remain open struggle to stock a full range of birth control methods,²⁵ access to contraceptive care will continue to dwindle, affecting millions of women.²⁶ In short, *Dobbs* jeopardized the viability of family planning clinics and, consequently, the provision of contraception.

The holding in *Dobbs* also undermined access to contraception by allowing its legality to be tarnished by association. Because the Court allowed states to ban abortion without providing definitional clarity, it enabled the inaccurate conflation of abortion (which ends a pregnancy at some point after the embryo

beneficiaries from receiving care at Planned Parenthood clinics by denying reimbursements); Carter Sherman, *Supreme Court Paves Way for South Carolina and Other States to Defund Planned Parenthood*, GUARDIAN (June 26, 2025, 11:22 EDT), <https://perma.cc/4J5L-UCHC> (observing that the decision could “embolden red states across the country to effectively ‘defund’ the reproductive healthcare organization”); Shefali Luthra, *Most Planned Parenthood Patients Are Insured Through Medicaid. The Supreme Court Just Undercut That*, 19TH NEWS (June 26, 2025, 7:18 AM PT), <https://perma.cc/KV4Y-U6EV> (noting that—in addition to South Carolina—Arkansas, Missouri and Texas disqualify Planned Parenthood from receiving Medicaid funds); Celine Castronuovo, *States Seize on Planned Parenthood as Trump Cuts Take Effect*, BLOOMBERG L. (updated Oct. 3, 2025, 8:48 AM PDT), <https://perma.cc/F4QR-3QK2> (“State lawmakers are forging ahead with proposals to disenroll Planned Parenthood from their Medicaid programs, emboldened by the Trump administration’s federal cuts and the US Supreme Court’s defense of states’ right to legislate on the issue.”).

22. Wells, *supra* note 15.

23. Jennifer J. Frost, Rachel Benson Gold & Amelia Bucek, *Specialized Family Planning Clinics in the United States: Why Women Choose Them and Their Role in Meeting Women’s Health Care Needs*, 22 WOMEN’S HEALTH ISSUES 519, 519 (2012).

24. See *supra* note 14 and accompanying text.

25. See Kaitlin Sullivan, *Cost of Stocking Birth Control Strains Family Planning Clinics in Some States*, NBC NEWS (Aug. 3, 2024, 3:00 AM PDT), <https://perma.cc/LV5M-R49F> (explaining that it “has become increasingly difficult” for family planning clinics in some states to supply a variety of contraceptives).

26. See Frost et al., *supra* note 23, at 519 (noting that, before *Dobbs*, more than seven million women received contraceptive care from publicly funded family planning clinics).

has been implanted) and contraception (which prevents fertilization or implantation of the embryo), thereby endangering the availability of several highly effective methods of birth control. Confusion about what counts as an abortifacient—often sown strategically²⁷—renders certain forms of contraception legally suspect.

This confusion predated *Dobbs*, fueling earlier debates around the classification of emergency contraception²⁸ and the Affordable Care Act's contraceptive mandate.²⁹ But with the post-*Dobbs* surge of restrictions and outright bans, abortion's stark illegality has newly tainted contraception.³⁰ The false equation of abortion with certain contraceptive methods—primarily emergency contraception³¹ and IUDs³²—consigns these forms of contraception to uncertain legal status. Those articulating this view include Trump administration officials,³³ university officers,³⁴ hospital administrators,³⁵ the

27. Cf. Kiera Butler, *The Anti-Abortion Movement's Next Target: Birth Control*, MOTHER JONES (May 5, 2022), <https://perma.cc/33FE-2ANT> (describing the anti-abortion movement's "ambitious future agenda" that involves "expanding the definition of abortion itself to include contraception"). See *infra* notes 68-69 and accompanying text.

28. Rachel Frank, Note, *Miss-Conceptions: Abortifacients, Regulatory Failure, and Political Opportunity*, 129 YALE L.J. 208, 211-12 (2019).

29. In the decade-plus since the Affordable Care Act's passage, a good deal has been written on the contraceptive mandate. See, e.g., Caroline Mala Corbin, *The Contraception Mandate*, 107 NW. U.L. REV. 1469, 1470-71 (2013). For a discussion of recent trends related to religious and moral objection, see notes 57-65 and accompanying text below.

30. See Elyssa Spitzer, *Defining and Defending Contraception Post-Roe*, CTR. FOR AM. PROGRESS (July 12, 2022), <https://perma.cc/N8GQ-CL9F>.

31. See *Practice Bulletin Number 152: Emergency Contraception*, 126 AM. COLL. OBSTET. & GYNECOL., Sept. 2015, at 1, 2 ("Emergency contraception sometimes is confused with medical abortion. Medical abortion is used to terminate an existing pregnancy, whereas emergency contraception is effective only before a pregnancy is established. Emergency contraception can prevent pregnancy after sexual intercourse and is ineffective after implantation.").

32. See *Practice Bulletin Number 186: Long-Acting Reversible Contraception Implants and Intrauterine Devices*, 130 AM. COLL. OBSTET. & GYNECOL., Nov. 2017, at 251, 252 (noting that "[t]he available evidence supports that [an IUD] does not disrupt pregnancy and is not an abortifacient").

33. See Stephanie Nolen, Jeanna Smialek & Edward Wong, *\$10 Million in Contraceptives Have Been Destroyed on Orders from Trump Officials*, N.Y. TIMES (updated Sept. 12, 2025), <https://perma.cc/WAN8-KBC2> (describing the government's destruction of birth control pills, IUDs, and hormonal implants, and quoting the explanation of a USAID spokesperson, who indicated that "[t]he administration will no longer supply abortifacient birth control under the guise of foreign aid").

34. See, e.g., Oriana González, *Post-Dobbs Birth Control Fight Heads to College Campuses*, AXIOS (updated Oct. 2, 2022), <https://perma.cc/MD6K-JJUJ> (describing a decision by administrators at the University of Idaho that, to comply with state abortion law in the wake of *Dobbs*, it would not provide students with contraception).

35. See, e.g., Jonathan Shoreman, *Kansas City Area Health System Stops Providing Plan B in Missouri Because of Abortion Ban*, KAN. CITY STAR (updated July 1, 2022, 12:36 AM), *footnote continued on next page*

authors of Project 2025,³⁶ and state³⁷ and federal³⁸ lawmakers. Even some medical providers have expressed uncertainty about whether contraception is implicated by their state's abortion ban.³⁹

In states with the most restrictive abortion policies, a recent study found a marked decline after *Dobbs* in prescriptions filled for contraceptive pills (the preferred form of hormonal contraception).⁴⁰ Other research has likewise shown a post-*Dobbs* reduction in access to a range of effective, non-permanent methods of birth control.⁴¹ Notwithstanding clear scientific distinctions

<https://perma.cc/T3R2-WTH6> (reporting on a leading health care system's announcement that it would no longer provide emergency contraception due to the state's abortion ban).

36. See DAVID A. GRAHAM, *THE PROJECT: HOW PROJECT 2025 IS RESHAPING AMERICA* 51 (2025) (noting that Project 2025 seeks a ban on emergency contraception, which it describes as a “potential abortifacient”).
37. See, e.g., Cat Ross, *South Carolina Is Trying to Apply Racketeering Laws to Criminalize Abortion Providers*, MS. (Sept. 25, 2025), <https://perma.cc/Y6M5-DBLZ> (explaining that in addition to its near-total abortion ban, the state bill under consideration would amend the definition of legal contraception to exclude anything that prevents ovulation). For example, one state's redefinition implicates “certain types of IUDs and emergency contraceptives,” *id.*, as well as hormonal birth control. See S. 323, 2025 Gen. Assemb., 126th Sess. (S.C. 2025) (defining “[c]ontraceptive” as a “drug, device, or chemical that prevents conception” while further specifying that “[c]onception” means fertilization of an ovum by sperm”); see also Kimya Forouzan & Rosann Mariappuram, *Midyear 2024 State Policy Trends: Many US States Attack Reproductive Health Care, as Other States Fight Back*, GUTTMACHER (Dec. 16, 2024), <https://perma.cc/6M5E-3G39> (summarizing an Indiana law that was “amended to provide [Medicaid reimbursements] for only subdermal contraceptives, based on the false claim by anti-abortion groups that IUDs cause abortions”).
38. Riley Rogerson, *GOP Lawmakers Oppose the Abortion Pill and ‘Plan B.’ They Hardly Understand Either*, DAILY BEAST (Apr. 21, 2024, 12:08 AM EDT), <https://perma.cc/HL8L-TQNR>.
39. See Yasaman Zia et al., “I Am Putting My Fear on Them Subconsciously”: A Qualitative Study of Contraceptive Care in the Context of Abortion Bans in the U.S., 21 REPROD. HEALTH, art. 171, at 4-6 (2024), <https://perma.cc/679G-CJCL> (finding that providers of contraceptive care expressed “confusion and uncertainty” about the impact of abortion restrictions on their services).
40. See Dima M. Qato, Rebecca Myerson, Andrew Shooshtari, Jenny Guadamuz & Caleb Alexander, *Use of Oral and Emergency Contraceptives After the US Supreme Court’s Dobbs Decision*, 7 JAMA NETWORK OPEN e2418620, at 2, 5, 8 (2024), <https://perma.cc/T3QZ-EP2Z> (finding that “*Dobbs* was associated with declines in the use of [oral contraceptive prescriptions] in the most restrictive states with a full ban on abortion,” and that prescription fills for emergency contraceptives decreased by 65% in abortion-restrictive states).
41. See, e.g., Julia Strasser, Ellen Schenk, Qian Luo, Mandar Bodas, Maria Murray & Candice Chen, *Contraception Usage and Workforce Trends Through 2022*, 7 JAMA NETWORK OPEN e2460044, at 1 (2024), <https://perma.cc/N3RP-5P5L> (showing overall declines in medical and prescription claims for pills, patches, rings, injectables, IUDs, and implants); Megan L. Kavanaugh & Amy Friedrich-Karnik, *Has the Fall of Roe Changed Contraceptive Access and Use? New Research from Four US States Offers Critical*
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between abortion and contraception, attacks on the former are fast becoming attacks on the latter.

II. Competing Norms Prevail

It would be a mistake to understand current threats to contraception as yoked *only* to anti-abortion laws. A separate category of threat takes direct aim at contraceptive access by subordinating it to ascendant norms. Insofar as *Dobbs* destabilized the federal constitutional right to contraception, the decision has spurred these independent, though related, efforts.

Intensifying moves to target contraceptive access are underpinned by a pair of distinct normative hooks: parental rights and religious or moral objection. In recent years, these norms have gained cultural, political, and legal traction. For instance, a host of post-pandemic laws passed in the name of parental autonomy allow parents to control what their children learn in public schools in subjects that touch on race and LGBTQ issues.⁴² Separately, the Supreme Court has decidedly tilted the doctrine of religious freedom in the direction of believers.⁴³ With increased recognition of these norms comes new forms of contraceptive gatekeeping.

First, the expansion of parental rights catalyzes the spread of state parental consent laws. Prior to 2024, about half the states limited the circumstances under which a minor could obtain contraceptive services.⁴⁴ In the 2024 legislative session, lawmakers in six states introduced eight bills that would

Insight, 2 HEALTH AFFS. SCHOLAR, qxae016, at 2 (2024), <https://perma.cc/8KYF-NY9A> (finding that after *Dobbs*, in three of four states investigated, individuals reported significantly higher levels of delays or other trouble accessing their preferred contraceptive method).

42. See Mary Ziegler, Maxine Eichner & Naomi Cahn, *Retrenchment by Division: The New Politics of Parental Rights*, 123 MICH. L. REV. 669, 702-06 (2025).

43. See Richard Schragger, Micah Schwartzman & Nelson Tebbe, *Reestablishing Religion*, 92 U. CHI. L. REV. 199, 201 (2025) (“In the last few years, a new pattern has emerged in the doctrine of religious freedom. Religion must be treated equally with respect to public funding and other forms of government support. But with respect to burdensome government regulations, believers are entitled to special treatment in the form of exemptions or accommodations.”); see, e.g., *Kennedy v. Bremerton Sch. Dist.*, 142 S. Ct. 2407, 2415-16 (2022) (holding that the Free Exercise Clause protects the right of a high school football coach to pray on the field after games); *Our Lady of Guadalupe Sch. v. Morrissey-Berru*, 140 S. Ct. 2049, 2055 (2020) (holding that a ministerial exception to the First Amendment bars teachers from bringing employment discrimination claims).

44. *Minors’ Access to Contraceptive Services*, GUTTMACHER (Aug. 30, 2023), <https://perma.cc/Y4AF-AFY9>.

require young people to get parental permission to obtain birth control.⁴⁵ Idaho passed such a law.⁴⁶

Regardless of how states choose to regulate minors' access to contraception, Title X has long prohibited programs receiving federal funding from requiring parental consent.⁴⁷ But recently, in a challenge to Title X's parental notification prohibition, a U.S. district court in Texas held that the provision in question violated a father's state statutory right to consent to his child's receipt of contraceptive services and, even more sweepingly, his federal due process right to direct his child's upbringing.⁴⁸ Parting ways with courts that had rejected similar constitutional claims,⁴⁹ Judge Matthew Kacsmaryk insisted that states retain the ability to regulate contraception.⁵⁰ A more ominous passage cited Justice Thomas's concurring opinion in *Dobbs* while raising the prospect of a blanket contraceptive ban.⁵¹ Such a move would be inconsistent with *Carey v. Population Services International*, which held that the constitutional right to privacy prohibits states from imposing a ban on the distribution of contraceptives to minors.⁵² But in Judge Kacsmaryk's view, this

45. See Forouzan & Mariappuram, *supra* note 37. In early 2025, a Florida lawmaker was the latest to introduce similar legislation. See Ana Goñi-Lessan, *Florida Teens May Be Blocked from Birth Control, STI Treatment Without Parental Consent*, TALLAHASSEE DEMOCRAT (updated Apr. 4, 2025, 10:56 AM ET), <https://perma.cc/R4LT-VW6C>.

46. See S.B. 1329, 67th Leg., 2d Reg. Sess. (Idaho 2024) (codified at IDAHO CODE § 32-1015); see also TENN. CODE ANN. § 36-8-103 (2024).

47. 42 C.F.R. § 59.10(b) (2025); see also WEN W. SHEN, CONG. RSCH. SERV., LSB10916, TITLE X PARENTAL CONSENT FOR CONTRACEPTIVE SERVICES LITIGATION: OVERVIEW AND INITIAL OBSERVATIONS (PART 1 OF 2) 1 (2023), <https://perma.cc/4WMP-LA7C> (observing that, in line with nearly four decades of cases from lower courts (including the District of Columbia, Second, Eighth, and Tenth Circuits), "current Department of Health and Human Services (HHS) regulations . . . prohibit Title X projects from requiring parental consent and notification for services provided to minors").

48. See *Deanda v. Becerra*, 645 F. Supp. 3d 600, 628-29 (N.D. Tex. 2022), *aff'd in part and rev'd in part*, 96 F.4th 750 (5th Cir. 2024).

49. See, e.g., *Anspach ex rel. Anspach v. City of Philadelphia Dep't of Pub. Health*, 503 F.3d 256, 262-64 (3d Cir. 2007); *Doe v. Irwin*, 615 F.2d 1162, 1168 (6th Cir. 1980). These rulings accord with *Carey v. Population Servs. Int'l*, 431 U.S. 678, 694 (1977) (plurality opinion).

50. See *Deanda*, 645 F. Supp. 3d at 623 n.12 ("That the constitutionally protected right of privacy extends to an individual's liberty to make choices regarding contraception does not, however, automatically invalidate every state regulation in this area." (quoting *Carey*, 431 U.S. at 685-86)).

51. *Id.* ("The correctness of *Carey's* holding on blanket bans of contraceptives is also in doubt insofar as it relies on *Eisenstadt v. Baird*, 405 U.S. 438 (1972), and *Griswold v. Connecticut*, 381 U.S. 479 (1965)." (citing *Dobbs*, 142 S. Ct. at 2301 (Thomas, J., concurring))).

52. *Carey*, 431 U.S. at 694.

holding is now “in doubt” because of its reliance on the seminal cases guaranteeing a right to birth control.⁵³

In March 2024, the Fifth Circuit affirmed the district court’s declaration that Title X does not preempt Texas’s parental consent law.⁵⁴ As a result, Texas teens now need parental consent to access contraceptive services at federally funded clinics.⁵⁵ These developments portend a next phase in the movement to place contraceptive care out of the reach of teens, not only in states that currently require parental consent, but—should the constitutional arguments eventually succeed—throughout the nation.⁵⁶

A second set of normative claims for restricting access to contraception is grounded in religious or moral objection.⁵⁷ For over a decade, courts have struggled to define the contours of an employer’s right to exclude contraceptive coverage from its health plan.⁵⁸ While the latest iterations of this conflict continue to pit religious freedom against contraception, the opposition has broadened and become more galvanized.⁵⁹ Pharmacists are refusing to fill prescriptions for hormonal birth control and emergency contraception.⁶⁰ Catholic hospitals are declining to offer certain forms of

53. *Deanda*, 645 F. Supp. 3d at 623 n.12.

54. *Deanda v. Becerra*, 96 F.4th 750, 753 (5th Cir. 2024). The Fifth Circuit also affirmed the district court’s standing analysis but reversed its decision to vacate a regulation prohibiting Title X grantees from obtaining parental consent, if only because the regulation was promulgated after the plaintiff filed suit. *Id.* The appellate court declined to reach the lower court’s holding that Title X violates a parent’s constitutional right to direct the child’s upbringing. *Id.* at 753, 766.

55. See Mishka Espey, *5th Circuit Ruling Will Almost Certainly Lead to More Teen Pregnancies, Says CAPS Sabrina Talukder*, CTR. FOR AM. PROGRESS (Mar. 12, 2024), <https://perma.cc/XV38-MCG2> (“[T]oday’s ruling strips youth of their sexual and reproductive autonomy and will almost certainly lead to more teen pregnancies in Texas—a state with some of the most restrictive sex education and abortion laws and, consequently, some of the highest teen pregnancy rates in the country.”).

56. See *Deanda*, 645 F. Supp. 3d at 628 (holding that “the right of parents to consent to their minor children’s use of contraceptives is deeply rooted in this Nation’s history and tradition,” and further, that “no compelling governmental interest justifies Defendants’ disregard of Plaintiff’s parental rights”).

57. These objections can, but need not, stem from the conflation of certain forms of contraception with abortion. See *supra* notes 27-39 and accompanying text.

58. See *Burwell v. Hobby Lobby Stores, Inc.*, 573 U.S. 682, 736 (2014); *Little Sisters of the Poor Saints Peter and Paul Home v. Pennsylvania*, 140 S. Ct. 2367, 2372 (2020).

59. See Anna Bernstein, Amy Friedrich-Karnik & Samira Damavandi, *How Project 2025 Seeks to Obliterate Sexual and Reproductive Health and Rights*, GUTTMACHER (Oct. 2024), <https://perma.cc/JNF5-Z565> (describing Project 2025 plans for “broadening the contraceptive coverage guarantee’s existing religious and moral exemptions to make it easier for any employer—including large, for-profit corporations—to exclude contraceptive coverage from their employees’ health plan”).

60. See *Pharmacy Refusals 101*, NAT’L WOMEN’S L. CTR., (July 14, 2025), <https://perma.cc/N4F8-VFET>. For an overview of the regulatory landscape, see Thomas S. Achey & *footnote continued on next page*

contraceptive care.⁶¹ Lawmakers are introducing new laws authorizing medical professionals not to provide any service that “violates the conscience.”⁶² One governor has repeatedly vetoed legislation that would protect the right to contraception because the bill lacks a “conscience clause.”⁶³

These moves align with an evident commitment on the part of the Trump administration to hinder contraceptive access in the name of religious freedom.⁶⁴ As this issue wends its way through the courts, several Justices have already indicated a willingness to leverage free exercise norms to defeat contraceptive access.⁶⁵

III. From Misinformation to Regulation

We now come to a final kind of threat: the spread of misinformation to promote birth control skepticism—a trend that augurs future regulation under the guise of protecting women.

In July 2023, the first over-the-counter oral contraceptive was approved.⁶⁶ Around this time, an influential segment of online culture that prominently

Ashley T. Robertson, *Conscientious Objection: A Review of State Pharmacy Laws and Regulations*, 57 HOSP. PHARM. 268, 270 (2022).

61. Maria Godoy, *Getting Contraception Gets Complicated for Patients at Catholic Hospitals*, NPR (Aug. 29, 2022, 5:09 AM ET), <https://perma.cc/B3LA-DWA9>; Yuan Liu, Luciana E. Hebert, Lee A. Hasselbacher & Debra B. Stulberg, “Am I Going to Be in Trouble for What I’m Doing?”: *Providing Contraceptive Care in Religious Health Care Systems*, 51 PERSPS. ON SEXUAL & REPROD. HEALTH 193, 193 (2019), <https://perma.cc/VH9H-U3H2>.

62. See TENN. CODE ANN. § 63-1-903 (2025). For ethical considerations, see COMM. ON ETHICS, *The Limits of Conscientious Refusal in Reproductive Medicine*, Committee Opinion No. 385, AM. COLL. OBSTET. & GYNECOL. 1 (Nov. 2007).

63. See Jessica F. Simmons, *Virginia Gov. Glenn Youngkin Vetoes Birth Control Protections . . . Again*, MS. (May 29, 2025), <https://perma.cc/H5PH-7UGH>.

64. See Anna Bernstein, Amy Friedrich-Karnik & Samira Damavandi, *10 Reasons a Second Trump Presidency Will Decimate Sexual and Reproductive Health*, GUTTMACHER (Nov. 2024), <https://perma.cc/P4LJ-53MB>; see also *Pennsylvania v. Trump*, No. 17-4540, 2025 WL 2349798, at *2 (E.D. Pa. Aug. 13, 2025) (granting summary judgment to states challenging the first Trump administration’s rules broadening religious and moral exemptions to contraceptive mandate). For different perspectives on the ballooning of free exercise claims, see Schragger et al., *Reestablishing Religion*, note 43 above, at 201; and John Witte, Jr., *A New Great Awakening of Religious Freedom in America*, 14 J. CHRISTIAN LEGAL THOUGHT, no. 1, 2024, at 44, 45.

65. See *Stormans, Inc. v. Wiesman*, 579 U.S. 942 (2016) (Alito, J., dissenting from denial of certiorari) (expressing skepticism about the state’s ability to require pharmacists with sincerely held objections to dispense contraception).

66. See *FDA Approves First Nonprescription Daily Oral Contraceptive*, FDA (Jul. 13, 2023), <https://perma.cc/LTK3-TPNU>.

features women promoting wellness began to turn against hormonal birth control.⁶⁷

The ensuing rise of birth control skepticism is driven by a coordinated campaign of moral opponents that relies in part on the contributions, however unwitting, of pro-choice influencers.⁶⁸ This campaign feeds largely on misinformation.⁶⁹ One researcher found that online discussions of birth control tend to “exaggerate side effects” and spread other misleading narratives, while “urging viewers to discontinue effective care and return to a more ‘natural’ state.”⁷⁰ Overwhelmingly, social media influencers are advising their millions of viewers to jettison hormonal contraception and replace it with *less effective* “fertility awareness-based methods”—those aimed at tracking menstrual cycles so as to avoid intercourse during fertile windows and thus prevent pregnancy.⁷¹

67. See Christine Emba, *Stop Shouting Down the Women Going off the Pill*, ATLANTIC (May 24, 2024), <https://perma.cc/J7MP-R3ZP> (“For the past few years, the ‘Why women are going off the pill’ essay has become a staple of lifestyle journalism. A search for birth control on TikTok yields thousands of videos, many taking a negative stance on hormonal methods.”).

68. See Kiera Butler, *Inside Anti-Abortion Groups’ Campaign to Sell Women on Unreliable Birth Control “Alternatives,”* MOTHER JONES (Sept. 30, 2022), <https://perma.cc/9ZMM-XDBJ> (observing that “[a]nti-abortion activists—many of whom are morally opposed to the idea of contraception because they consider it a form of abortion or just morally wrong—have found that wellness influencers, many of them pro-choice, are a boon to their cause”). Since 2021, more than \$500 million in state funds have been directed to religiously affiliated crisis pregnancy centers and “alternatives to abortion” programs that “spread misinformation about birth control and discourage its use.” Madison Pauly, *What Would the Future of Birth Control Be Under Trump? Ask Texas*, MOTHER JONES (Oct. 31, 2024), <https://perma.cc/EU28-LDWV>. The rise of birth control skepticism is part of a larger cultural turn against science. See generally PETER HODETZ, *THE DEADLY RISE OF ANTI-SCIENCE: A SCIENTIST’S WARNING* 1-13 (2023) (describing the rise of anti-science attitudes across parts of the United States).

69. Lauren Weber & Sabrina Malhi, *Women Are Getting Off Birth Control Amidst Misinformation Explosion*, WASH. POST (March 21, 2024), <https://perma.cc/3HDL-5RJJ>. For an overview, see Naseem S. Miller, *How Influencers and Content Creators Discuss Birth Control on Social Media: What Research Shows*, JOURNALIST’S RES. (May 15, 2024), <https://perma.cc/U88Z-3UNV>.

70. Emily Pfender, Opinion, *Social Media Misinformation Is Scaring Women About Birth Control*, STAT NEWS (Nov. 5, 2024), <https://perma.cc/2DZP-DFLX>; Emily J. Pfender & M. Marie Devlin, *What Do Social Media Influencers Say About Birth Control? A Content Analysis of YouTube Vlogs About Birth Control*, 38 HEALTH COMM’N 3336, 3336 (2023); see also Caroline de Moel-Mandel, Arielle Donnelly & Megan Bugden, “Do You Know What Birth Control Actually Does to Your Body?”: Assessing Contraceptive Information on TikTok, 57 PERSPS. ON SEXUAL & REPROD. HEALTH 358, 359 (2025).

71. See *Fertility Awareness*, PLANNED PARENTHOOD, <https://perma.cc/Y327-BM6U> (archived Oct 25, 2025) (discussing various fertility awareness methods and noting that they “don’t work as well as other types of birth control”); see also Dennis Thompson, *More Abortions Tied to Natural Family Planning or Fertility Awareness Methods*, U.S. NEWS & WORLD REP. (Jan. 14, 2025), <https://perma.cc/D5T7-7SL2>.

Many women are taking this message to heart. Research suggests that engagement with birth control skepticism tends to reduce contraceptive usage.⁷² Put differently, the anti-contraceptive message is resonating. For many women, particularly those with marginalized identities, poor treatment of their communities by the medical profession has engendered understandable mistrust.⁷³ And hormonal birth control may indeed have side effects that make other contraceptive methods a better choice for some users.⁷⁴ Against this backdrop, a cultural movement that defies medical consensus—even if largely designed to push women to forsake effective contraception⁷⁵—is bound to find a receptive audience, however misleading its factual tenets.⁷⁶

Beyond shaping women's birth control preferences, the “tidal wave of backlash”⁷⁷ against hormonal contraceptives is readily translatable into governmental policy.⁷⁸ Already this backlash is finding an outlet at the federal agency level.⁷⁹ In January 2025, the Trump administration removed the

72. See Corinne R. Rocca, Isabel Muñoz, Lavanya Rao, Sara Levin, Ori Tzvieli & Cynthia C. Harper, *Measuring a Critical Component of Contraceptive Decision Making: The Contraceptive Concerns and Beliefs Scale*, 28 MATERN. & CHILD HEALTH J. 847, 852-54 (2024); Emily J. Pfender & Leah R. Fowler, *Social Media Is Influencing Contraceptive Choice*, 33 J. WOMEN'S HEALTH 563, 563 (2024).

73. See Ivy K. Ho, Taylor A. Sheldon & Elliott Botelho, *Medical Mistrust Among Women with Intersecting Marginalized Identities: A Scoping Review*, 27 ETHN. & HEALTH 1733, 1734 (2022); see also Paul Hudson & Michelle A. Williams, *People Are Much Less Likely to Trust the Medical System If They Are from an Ethnic Minority, Have Disabilities, or Identify as LGBTQ+*, *According to a First-of-Its-Kind Study by Sanofi*, FORTUNE (Jan. 31, 2023 7:30 AM EST), <https://perma.cc/ZF7W-HRS8>.

74. *Facts Are Important: Hormonal Birth Control*, AM. COLL. OBSTETRICIANS & GYNCOLOGISTS (May 2024), <https://perma.cc/2DPJ-738Y>; *Birth Control Pill FAQ: Benefits, Risks and Choices*, MAYO CLINIC (May 10, 2023), <https://perma.cc/RHS8-VB2K>.

75. See *supra* note 68 and accompanying text.

76. This misinformation tends to exaggerate the side effects (both incidence and severity) of hormonal birth control while overstating the effectiveness of fertility awareness methods. See *supra* notes 69-71 and accompanying text.

77. See Butler, *Inside*, *supra* note 68.

78. See *infra* notes 79-92 and accompanying text; Usha Ranji, Laurie Sobel & Alina Salganicoff, *A New Reproductive Health Landscape? Possible Actions that Could Be Undertaken During the Second Trump Administration*, KFF (Dec. 19, 2024), <https://perma.cc/V45W-QRA9> (explaining that Project 2025 calls for shifting Title X contraceptive coverage to fertility awareness methods). The concerted push for increased reliance on period- and fertility-tracking technologies is even greater cause for concern in a post-*Dobbs* era of heightened surveillance. See Leah R. Fowler & Michael R. Ulrich, *Femtechnodystopia*, 75 STAN. L. REV. 1233, 1272-75 (2023).

79. Policies related to contraception serve the administration's larger agenda to “reverse declining birthrates and push conservative family values.” Caroline Kitchener, *White House Assesses Ways to Persuade Women to Have More Children*, N.Y. TIMES (Apr. 21, 2025), <https://perma.cc/8LYT-LGQC>. This agenda envisions childbearing as part of a woman's natural and civic duty, thereby casting the decision to use birth control as morally questionable and paving the way for further restrictions.

Department of Health and Human Services website (reproductiverights.gov) that provided basic reproductive health-related information, including the availability of coverage for contraception under the Affordable Care Act.⁸⁰ In April, the administration fired a team of scientists responsible for producing and updating “the bible of contraceptive care.”⁸¹ And in May, President Trump nominated an avowed opponent of hormonal birth control to serve as the Surgeon General.⁸² All told, these moves reflect what is aptly described as a troubling “loss of evidence-based contraceptive care”⁸³—a loss quite likely to accelerate in the coming years.⁸⁴

Over time, misinformation around contraception can be used to justify any number of regulatory measures, up to and including bans. This playbook has been used time and time again.⁸⁵ Under the veneer of protecting women, policymakers distort the evidence or draw on junk science to restrict access to

80. Lauren Fichten, *Government Website Offering Reproductive Health Information Goes Offline*, CBS NEWS (Jan. 20, 2025, 10:05 PM EST), <https://perma.cc/6C5W-X8EJ>. The website remains offline as of October 29, 2025.

81. See Meredith Wadman, *CDC Team That Updated Go-To Physician Reference on Contraceptive Science Is Fired*, SCI. (Apr. 15, 2025, 6:20 PM ET), <https://perma.cc/5RCG-GQAX> (“The dismissals throw into jeopardy the future of the key reference[, the U.S. Medical Eligibility Criteria for Contraceptive Use,] that health care providers rely on to guide patients to the safest and most effective contraceptives.”). Existing guidelines will soon become obsolete. See Katia Riddle, *After CDC Cuts, Doctors Fear Women Will Lose Access to Contraception Research*, NPR (May 28, 2025, 1:14 PM ET), <https://perma.cc/GY86-9XXV> (reporting on concerns that doctors will be unable to compensate for the absence of CDC updates).

82. Ryan Foley, *4 Things to Know About Trump’s New Surgeon General Pick Dr. Casey Means*, MSN (May 14, 2025), <https://perma.cc/Y359-AXPW>. In a 2024 interview with Tucker Carlson, Casey Means, the nominee, expressed concern that birth control is “literally shutting down the hormones in the female body that create this cyclical, life-giving nature of women.” *Id.* Means further criticized efforts to “shut it down [women’s cycles and reproduction],” and denounced those who would suggest “there is no repercussions for that [taking birth control].” *Id.* “By taking a hammer to women’s hormones,” Means added, “what we’ve done is we are destroying the things that give us life in this country.” *Id.* For context, the Surgeon General is responsible for “bring[ing] the best available scientific information to the public.” *About the Office of Surgeon General*, U.S. DEP’T HEALTH & HUM. SERVS., <https://perma.cc/5Z47-S6PM> (archived Oct. 25, 2025).

83. Preetha Nandi & Angeline Ti, *Yet Another Political Blow: The Loss of Evidence-Based Contraceptive Care*, MEDPAGE TODAY (May 3, 2025), <https://perma.cc/4TF8-LTNE>; see also Riddle, *After CDC Cuts*, *supra* note 81.

84. On the politics of birth control skepticism, see Jennifer Gerson & Mariel Padilla, *A New Generation of Birth Control Skeptics Leans Right*, 19TH NEWS (May 14, 2025, 11:34 AM PT), <https://perma.cc/89CN-BFN7>.

85. Anti-abortion regulation has long drawn on dubious scientific claims. See, e.g., David Robert Grimes, *A Scientist Weighs Up the Five Main Anti-Abortion Arguments*, GUARDIAN (Aug. 12, 2015), <https://perma.cc/2GU6-FW83> (discussing how leading anti-abortion arguments are at odds with scientific consensus).

reproductive care. Once skepticism is entrenched and the surrounding evidentiary baseline degraded, legal restrictions become far more actionable.

In the past, government officials have deployed a range of regulatory approaches to operationalize a false or exaggerated narrative of danger to women and gender minorities, including labeling requirements,⁸⁶ counseling mandates,⁸⁷ design “safeguards,”⁸⁸ administrative clawbacks framed as safety controls,⁸⁹ and selected prohibitions targeting minors.⁹⁰

Given its precarious cultural status, hormonal birth control has become an easy next target for this treatment. Indeed, opponents have recently called for greater “medical safeguards” around contraception,⁹¹ and they are encouraging the revocation of FDA approval for over-the-counter birth control pills.⁹²

For now, the idea of a ban on hormonal contraception may seem far-fetched. But recent federal actions point to a regulatory pipeline that flows from cultural suspicion to legal restriction. As distrust builds and misinformation proliferates, a more muscular regulatory regime can emerge. The foundation is being laid.

86. Press Release, *FDA Responds to Evidence of Possible Association Between Autism and Acetaminophen Use During Pregnancy*, FDA (Sept. 22, 2025), <https://perma.cc/W4AV-2WRD>; see *ACOG Affirms Safety and Benefits of Acetaminophen During Pregnancy*, AM. COLL. OBSTETRICIANS & GYNECOLOGISTS (Sept. 22, 2025), <https://perma.cc/9AY4-BHAT>.

87. Talia Curhan, *Counseling and Waiting Period Requirements for Abortion*, GUTTMACHER (July 7, 2025), <https://perma.cc/75WR-H2QU>.

88. Kimya Forouzan, *Targeted Regulation of Abortion Providers*, GUTTMACHER (Aug. 13, 2025), <https://perma.cc/E25E-4U5K>.

89. Nathaniel Weixel & Joseph Choi, *Trump Administration Reviewing Abortion Pill*, HILL (Sept. 24, 2025, 6:15 PM ET), <https://perma.cc/WT2H-JJEW>; Shefali Luthra, *Abortion Opponents Are Coming for Mifepristone Using What Medical Experts Call “Junk Science,”* 19TH NEWS (May 20, 2025, 10:47 AM PT), <https://perma.cc/8REL-DN67>.

90. Stacey Weiner, *States Are Banning Gender-Affirming Care for Minors. What Does That Mean for Patients and Providers?*, AM. ASS’N MED. COLLS. (Feb. 20, 2024), <https://perma.cc/K85Z-2TDP>.

91. See Alice Miranda Ollstein & Megan Messerly, *Trump Says He Won’t “Ban” Birth Control. Here’s What He May Do Instead*, POLITICO (updated May 29, 2024, 1:16 PM EDT), <https://perma.cc/FXB6-MGY7> (quoting a senior researcher at the Heritage Foundation who wants “to see the National Institutes of Health or another entity study the long-term effects of birth control,” criticizes “an absolute right to contraception,” and views anti-contraception policies “not as ‘restrictions,’ but rather ‘medical safeguards’ for women”).

92. See, e.g., Michael J. New, *No, Over-the-Counter Birth Control Will Not Lower Abortion Rates*, NAT’L REV. (July 16, 2023, 7:17 PM), <https://perma.cc/9QZP-9JCM> (entreating conservatives to “ask future Republican presidential administrations to revoke FDA approval for . . . over-the-counter contraceptives”).

IV. Mapping the Threat Landscape

The three categories of ongoing contraceptive threats that I have identified constitute a unified movement—one that has gone largely unnoticed. Chief among the reasons for this neglect is that birth control opponents seldom take explicit aim at their target. Instead, access is reduced indirectly by clinic defunding, definitional blurring, parental and conscience gatekeeping, and misinformation-primed regulation. All the while, the movement to restrict contraception gains power by each of these individual pathways ratifying and reinforcing one another.

The false equation of certain birth control methods with abortion strengthens religious freedom claims for regulation while helping to legitimize the closure of clinics providing contraceptive care. At the same time, misinformation campaigns create unwarranted fears about birth control, whittling away opposition to regulation and bolstering parental rights objections. With the spread of a definitional fog around contraception—its workings, its availability, its safety—regulation can become more commonplace.

Together, these dynamics hasten the jurisprudential demise of a constitutional contraceptive right, which in turn enables ever more draconian measures by state and federal lawmakers to curtail access. Seemingly unrelated developments are acting in concert, with potential cumulative effects that are staggering.

Conclusion

Amidst disclaimers and protestations to the contrary, birth control opponents are working to dismantle an increasingly vulnerable system of access. The movement against contraception crystallizes the limits of extant protections along with the stakes of losing them altogether. While these attacks will not abate any time soon, they can be contested.

To defend against contraceptive threats that are bound to escalate, those committed to maximizing reproductive agency⁹³ must first recognize the nature of the ongoing assault. Opponents of contraception are simultaneously pursuing multiple reinforcing approaches to restrict availability. And because birth control is broadly popular, the success of this movement depends on the concealment of its ultimate design. Uncovering the mechanisms that incrementally erode access is thus a necessary precondition for resistance.

93. See Tuerkheimer, *Unwanted Pregnancy*, *supra* note 12 (manuscript at 56-61) (discussing the meaning of reproductive agency).

In this moment of precarity, contraception defenders also have an opportunity—not only to shore up a right to birth control, but to translate this right to universal access. In sixteen states and D.C., the right to contraception is protected by law or, better still, constitutional provision.⁹⁴ A variety of policy levers can be used to expand availability, including stringent insurance requirements, scope of practice modifications that facilitate dispensation, and eased pathways for telehealth distribution.⁹⁵ Beyond the removal of physical impediments and barriers to affordability, meaningful access requires that accurate contraceptive information is widely disseminated.⁹⁶

The ongoing siege offers new reason to commit to the unrealized promise of contraceptive availability for all. Success will require constant vigilance against emboldened opposition to what not long ago seemed given.

94. See Mabel Felix, Laurie Sobel & Alina Salganicoff, *The Right to Contraception: State and Federal Actions, Misinformation, and the Courts*, KFF (May 23, 2024), <https://perma.cc/R7AF-CR3J> (listing fourteen states—California, Colorado, Florida, Illinois, Massachusetts, Michigan, Minnesota, New Jersey, New Mexico, Ohio, Oregon, Rhode Island, Vermont, and Washington—plus D.C., with legal or constitutional protections for the right to contraception). In addition, Maryland and Missouri recently amended their respective constitutions to protect the right to contraception. See Danielle J. Brown, *Maryland Votes to Enshrine Reproductive Freedom in State Constitution*, MD. MATTERS (Nov. 5, 2024, 10:30 PM) (enshrining “an individual’s fundamental right to reproductive freedom, including but not limited to the ability to make and effectuate decisions to prevent, continue or end the individual’s pregnancy”); Gabrielle Hays, *Missouri Voters Approve Measure to Protect Abortion Rights*, PBS (Nov. 5, 2024, 11:52 PM EDT) (recognizing a “fundamental right to reproductive freedom” that includes contraception and abortion). The Missouri amendment is currently under attack by Republican lawmakers. See David A. Lieb, *Missouri Lawmakers Pass Referendum Seeking to Repeal Abortion-Rights Amendment Approved by Voters Last Year*, PBS (May 15, 2025, 2:22 PM EDT), <https://perma.cc/76E2-KA4C>.

95. See *State Contraception Policies*, NAT’L CONF. STATE LEGIS. (updated Sept. 29, 2025), <https://perma.cc/3MVV-EH8P> (listing states that have enacted each of these policies). Outside the policy realm, individuals can also organize to promote contraceptive access. See, e.g., Christine Fernando, *A Student “Womb Service” Works Covertly to Deliver Contraception at a Catholic College*, ASSOC. PRESS (updated Oct. 1, 2025, 11:03 AM PDT), <https://perma.cc/R4U7-C7LA> (reporting that “[a]t Catholic universities, which generally do not offer contraceptives on their campuses or at school-run health centers, student groups have stepped in to fill what they see as gaps in reproductive health care”).

96. Sex education in this country is an uneven patchwork. See Mollie Fairbanks, *Sex Education and HIV Education*, GUTTMACHER (Sep. 16, 2025), <https://perma.cc/ZVB9-4D2C> (reporting that less than half the states require the provision of information on contraception). On the popularity across the political spectrum of comprehensive sex education, see Leslie Kantor & Nicole Levitz, *Parents’ Views on Sex Education in Schools: How Much do Democrats and Republicans Agree?* 12 PLOS ONE e0180250, at 5-7, <https://perma.cc/E5ER-RBFC> (2017). See also Bernstein, *supra* note 59 (noting Project 2025’s recommendation to replace comprehensive sex education with abstinence-only curricula).